

Quality you can trust....

PATIENT'S NAME : Mr.A.ADARSH **AGE / SEX** : 39 Y / M
IDENTIFICATION NO. : 122232049 **DATE** : 02/06/2025
SAMPLE DATE : 02/06/2025 07:47:27 **REPORT DATE** : 02/06/2025 13:56:54
SAMPLE SOURCE :

TEST NAME	RESULT	UNIT	BIOLOGICAL REF. INTERVAL
Plasma Sugar Fasting Methodology : Hexokinase	101	mgs/dl	70 - 110
Serum Urea Methodology : Urease/GLDH	30	mg/dl	17 - 43
Blood Urea Nitrogen Methodology : Urease/GLDH	14	mg/dl	7 - 21
Serum Creatinine Methodology : Kinetic Colorimetric (Jaffe) assay	0.94	mg/dl	0.70 - 1.20
Serum Calcium Methodology : NM BAPTA	9.3	mg /dl	8.5 - 10.5
Serum Sodium Methodology : Indirect ISE	140	mEq/L	135 - 150
Serum Potassium Methodology : Indirect ISE	3.9	mEq/L	3.5 - 5.0
Serum Bicarbonate Methodology : Enzymatic carbonate	22	meq/L	22-26
Serum Chloride Methodology : Indirect ISE	104	mEq/L	96 - 104
Serum B12 Methodology : Chemiluminescence	374	pg/ml	180 - 914
Serum T3 Methodology : Eletro Chemiluminescence	1.0	ng/ml	0.8 - 2.3
Serum T4 Methodology : Eletro Chemiluminescence	7.9	ug/dl	4.8-12.0



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Serum Thyroid Stimulating Hormone Methodology : Eletro Chemiluminiscence	1.5	mIU/ml	0.35 - 5.5
Serum Total Prostate Specific Antigen Normal Range : 95% apparently healthy persons have values less than 4.0 ng/ml Methodology : Eletro Chemiluminiscence	1.05	ng/ml	0-4
Stool Occult Blood Methodology : Guaiac method	Negative		



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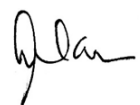
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TEST NAME	RESULT	UNIT	BIOLOGICAL REF. INTERVAL
Lipid profile			
Serum Cholesterol	192	mg/dl	Less than 200
Methodology : CHOD - POD			
Serum Triglycerides	52	mg/dl	Fasting 30 - 149
Methodology : Enzymatic Colorimetric Assay			
Serum H.D.L. Cholesterol	50	mg /dl	40 - 59
Methodology : Homogeneous Enzymatic Colorimetric Assay			
Serum L.D.L. Cholesterol	133	mg/dl	Less than 130
Methodology : Direct LDL(Homogeneous Enzymatic Colorimetric Assay)			
Serum V.L.D.L. Cholesterol	9	mg/dl	10 - 35
Methodology : Calculated value			

INTERPRETIVE INFORMATION*:

Total Cholesterol : Adults :- Desirable < 200 mg/dL;
 Border-line High 200 – 239 mg/dL;
 High > 239 mg/dL;
 Children :- Desirable < 170 mg/dL
 Border-line High 170 – 199 mg/dL
 High > 199 mg/dL;
 Triglycerides (Fasting) : Adults :-
 Border-line High 150 – 199 mg/dL
 High 200 - 399 mg/dL
 Very High > 400 mg/dL
 HDL Cholesterol : Desirable & Negative Risk factor for Coronary heart disease (CHD) > 60 mg/dL
 Risk factor for CHD < 40 mg/dL
 LDL Cholesterol : Treatment goals for patients based on Risk factors and history of CHD :-
 If no history of CHD and less than two Risk factors < 130 mg/dL
 Patients with low to moderate risk < 100 mg/dL
 Patients with high risk < 70 mg/dL

* Ref: NCEP ATP-III and Lipid Association of India 2020 Consensus statement

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TEST NAME	RESULT	UNIT	BIOLOGICAL REF. INTERVAL
Liver function test			
Serum Total Bilirubin	0.6	mgs/dl	0 - 1
Methodology : Diazo Method			
Serum Direct Bilirubin	0.2	mgs/dl	0-0.3
Methodology : Diazo Method			
Serum S.G.O.T.	23	U/L	0-45
Methodology : IFCC Kinetic			
Serum S.G.P.T.	31	U/L	0-37
Methodology : IFCC Kinetic			
Serum Alkaline Phosphatase	51	U/L	30 - 120
Methodology : PNPP Kinetic			
Serum Total Proteins	7.1	gms/dl	6.6 - 8.7
Methodology : Biuret			
Serum Albumin	4.4	gms/dl	3.5 - 5.0
Methodology : Bromocresol Green			
Serum Globulin	2.7	gms/dl	2.3-3.5
Methodology : Calculated Value			
A/G Ratio	1.63		More than 1.0
Methodology : Calculated Value			



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<u>TEST NAME</u>	<u>RESULT</u>	<u>UNIT</u>	<u>BIOLOGICAL REF. INTERVAL</u>
Glycosylated Haemoglobin			
Glycosylated Haemoglobin (HbA1c)	5.3	%	
Methodology : HPLC, Traceable to NGSP			
Estimated Average Glucose (eAG)	104	mg/dL	
Methodology : Calculated Value			

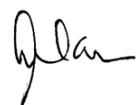
Interpretation of HbA1c Result:-

Pre-diabetes : 5.7 - 6.4

Diagnosing Diabetes : 6.5 or More

Goal for Glycaemic Control : Less than 7.0

Action Suggested : More than 8.0

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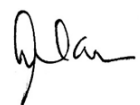
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<u>TEST NAME</u>	<u>RESULT</u>	<u>UNIT</u>	<u>BIOLOGICAL REF. INTERVAL</u>
. Serum 25-Hydroxy Vitamin D Methodology : CMIA	26	ng/ml	Refer to Interpretation

Interpretation:

Deficiency : Less than 20
 Insufficiency : 20- 29
 Optimum level : 30- 80
 Possible toxicity : More than 150

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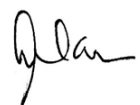
TEST NAME	RESULT	UNIT	BIOLOGICAL REF. INTERVAL
-			
Serum Hepatitis B surface antigen	Non Reactive(COI=0.40)	COI	Less than 0.9
Methodology : Electro Chemiluminescence			

Interpretation:-

INDEX	RESULT
-----	-----
More than 1.0	Reactive
0.9 to 1.0	Borderline
Less than 0.9	Non Reactive

Comments:-

- Hepatitis B Virus (HBV) is a member of the Hepadna virus family causing infections of the liver with extremely variable clinical features.
- Hepatitis B is transmitted primarily by body fluids especially serum and also spread effectively sexually and from mother to baby.
- In most individuals HBV hepatitis is self limiting, but 1-2% normal adolescents and adults develop Chronic Hepatitis.
- Frequency of chronic HBV infection is 5-10% in immunocompromised patients and 80% in neonates.
- The initial serological marker of acute infection is HBsAg which typically appears 2-3 months after infection and disappears 12-20 weeks after onset of symptoms.
- Persistence of HBsAg for more than six months indicates development of carrier state or Chronic liver disease.

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TEST NAME	RESULT	UNIT	BIOLOGICAL REF. INTERVAL
Complete blood counts (EDTA Blood)			
Total Leucocyte Count	6150	cells/cmm	4000 - 10000
Methodology : Electrical impedance/Cell counter			
Erythrocyte Count	4.50	millions/cmm	4.00 - 5.20
Methodology : Electrical impedance/ Cell counter			
WBC differential count : Neutrophils	32.5	%	40 - 80
Lymphocytes	55.3	%	35 - 45
Eosinophils	1.6	%	0 - 5
Monocytes	9.9	%	2 - 10
Basophils	0.7	%	0 - 2
Methodology : Flow cytometry/Cell counter			
Absolute Neutrophil Count	2000	cells/cmm	2000 - 7000
Methodology : Calculated .			
Absolute Lymphocyte Count	3400	cells/cmm	1000 - 3000
Methodology : Calculated ,			
Absolute Eosinophil Count	100	cells/cmm	20 - 500
Methodology : Calculated.			
Absolute Monocyte Count	610	cells/cmm	200 - 1200
Methodology, : Calculated			
Absolute Basophil Count	40	Cells/cmm	20 - 100
Methodology. : Calculated.			
Erythrocyte Sedimentation Rate	18	mm/1hour	0 - 12
Methodology : Modified Westergren			
Platelet Count	191	10 ^ 3 /cmm	150 - 410
Methodology : Electrical impedance/ Cell counter			
Mean Platelets Volume	10.0	fL	7.4 - 10.4
Methodology : Electrical impedance			
Haemoglobin	13.8	gms/dl	13 - 17
Methodology : Cell counter photometry SLS			
Hematocrit	39.2	%	40 - 50
Methodology : Cell Counter/Cumulative pulse height			



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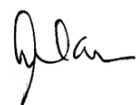
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TEST NAME	RESULT	UNIT	BIOLOGICAL REF. INTERVAL
Mean Corpuscular Volume Methodology : Calculated	87.1	femtolitres	83 - 101
Mean Corpuscular Haemoglobin Methodology : Calculated	30.7	pico-grams	27 - 32
Mean Corpuscular Haemoglobin Concentration Methodology : Calculated	35.2	%	31 - 37
Red Cell Distribution Width (CV) Methodology : Calculated	12.4	%	11.6 - 14.0

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TEST NAME	RESULT	UNIT	BIOLOGICAL REF. INTERVAL
Urine analysis			
Specific Gravity	1.025		1.005 - 1.030
Methodology : Refractometry			
Urine pH	5.0		4.8 - 7.4
Methodology : Reflectance Photometry Mixed Indicator			
Urine Glucose	Nil		Nil
Methodology : Reflectance Photometry GOD-POD			
Urine Ketone	Negative		Negative
Methodology : Reflectance Photometry Sodium Nitroprusside			
Urine Protein	Nil		Nil
Methodology : Reflectance Photometry Peroxidase			
Urine Nitrites	Negative		Negative
Methodology : Reflectance Photometry Modified Griess Reaction			
Urine Bile Salts	Negative		Negative
Methodology : Hay's test			
Urine Bilirubin	Negative		Negative
Methodology : Reflectance Photometry Diazonium in Acidic Medium			
Urine Urobilinogen	Normal (0.2)		0.2-1.0 EU/dl
Methodology : Reflectance Photometry Diazonium in Acidic Medium			
Urine Leucocyte	Negative		Negative(0-10/ul)
Methodology : Reflectance Photometry Esterase			
Urine Microscopy : Leucocytes	1 to 2	Cells/hpf	0 - 5
RBC	Nil	Cells/hpf	0 - 5
Epith. Cells	0 to 1	Cells/hpf	0 - 5
Casts	Nil		Nil
Crystals	Nil		Nil
Methodology : Digital Imaging and APR			



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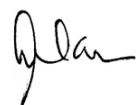
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Stool examination			
Stool : Ova	Nil		
Cysts	Nil		
Leucocytes per h.p.f.	1 to 2		
R.B.C. per h.p.f.	0 to 1		

Methodology : Microscopy

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<u>TEST NAME</u>	<u>RESULT</u>	<u>UNIT</u>	<u>BIOLOGICAL REF. INTERVAL</u>
Fecal Calprotectin	9	µg/g	
Methodology : CLIA			

Interpretation guidelines:-

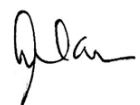
Normal (Up to 50 µg/g):Indicates no active inflammatory process in gastrointestinal system

Borderline (51 to 120 µg/g):Indicates mild inflammatory process such as NSAID or aspirin use, or a treated IBD. For patients of suspected IBD, retesting after 4 to 6 weeks is suggested.

High (>120 µg/g): Indicates an active inflammatory intestinal process and further diagnostic evaluation to determine aetiology is suggested.

Note: Elevated fecal calprotectin helps in distinguishing Inflammatory bowel disease (IBD) from functional gastrointestinal disorders such as Irritable bowel syndrome (IBS). It has a sensitivity and specificity of > 90% in the differential diagnosis of IBD and IBS.

The laboratory report must be interpreted in conjunction with the clinical profile of the patient by the clinician.

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